



PROCEDURE

Touchstone – Communicating For Safety

Scope (Staff):	All Staff
Scope (Area):	CAMHS Touchstone

CAHS commits to being a child safe organisation by applying the
[National Principles for Child Safe Organisations](#)

Read the full statement here:

[CAHS Child Safe Organisation Commitment Statement](#)

This document should be read in conjunction with this [disclaimer](#)

Aim

To clearly outline how risk and other safety concerns are communicated, documented and handed over within Touchstone CAMHS. This policy must be read in conjunction with the [CAHS Communicating for Safety Policy](#), CAMHS [Medical Records Management policy](#), [CAMHS Risk Assessment and Management Policy](#) and the [CAMHS MDT Clinical Reviews](#) guideline.

Risk

Failure to effectively communicate, document and handover information may comprise continuity of care and / or patient confidentiality.

Services may also be in breach of the Australian Standard 2828 Part One - Paper-Based Health Care Records (2012), the National Safety and Quality Health Service (NSQHS) (2nd Edition) Standard 6 (Communicating for Safety) , the Freedom of Information Act 1992 (WA), and the duty of the health service to fulfil the obligations of discovery of patient information in medico-legal cases.

General Principles – Risk Assessment and Management at Touchstone

- All risk assessment and management in Touchstone occurs as outlined in the CAMHS Risk Assessment and Management Policy.
- Each young person is to have a completed CAMHS Risk Assessment and Management Plan (CRAMP) with a documented safety plan on commencement of the program.

- The CRAMP is to be reviewed at any time the level of risk changes, the management/safety plan changes, and following any clinical incident involving the young person.

Procedure

‘Check in’ meeting

- The Check In meeting occurs each morning, once all young people have arrived at the Touchstone site for the commencement of the day program.
- The Check In meeting is attended by all of the young people who will be participating in the program that day.
- The meeting is attended and facilitated by two clinical staff members. All young people in attendance at the meeting are invited to ‘check in’ to the day. This is an opportunity for the facilitating staff to observe each young person’s presentation, level of engagement, and risk at that time.
- The observations, level of engagement, presenting risks and safety plan are to be documented in the integrated progress notes section of the young person’s medical record **during or immediately following** the Check In meeting.
- If the young person’s presentation at the Check In meeting indicates increased risk, the facilitating clinician will ensure that the young person’s CRAMP is updated to reflect the change in level of risk (see [CAMHS Risk Assessment and Management policy](#)). The facilitating clinician must also provide a verbal handover to the young person’s Case Manager, Touchstone Head of Service, or another Senior Staff Member.

Morning process meeting

- The Morning Process meeting occurs after the first three day program sessions (Check In, School, Individual therapy / Group) have concluded.
- The Morning Process meeting is attended by all members of the multidisciplinary team (MDT) who are available at the time of the meeting.
- A minimum of two clinicians is required for the meeting to proceed. Clinicians who are unable to attend the Morning Process meeting are expected to provide a handover to one (or more) of the clinicians who will be in attendance at the Morning Process meeting. This handover must occur prior to the commencement of the meeting.
- Staff in attendance at the Morning Process meeting must provide an update of each young person’s presentation following their participation in the first three day program sessions.
- For each of their clients, Case Managers are responsible for documenting these discussions in the medical record (if the Case Manager is not available, this task is assigned to another clinician).

- Individual therapy sessions, family therapy sessions, and Psychiatry reviews are documented separately by the clinician who facilitated the session. If deemed relevant by the facilitating clinician, each young person's presentation at these sessions may be discussed at the Morning Process meeting and documented in the medical record.
- Group therapy sessions are not documented separately by the clinician who facilitated the session. Each young person's presentation at these sessions **must** be discussed at the morning process meeting and documented in the medical record by the young person's case manager.
- If the above process indicates that the young person's level of risk has increased, the young person's CRAMP must also be updated - See CAMHS [Risk Assessment and Management policy](#).

Afternoon process meeting

- The Afternoon Process meeting occurs after the last day program session (Check out, Creative sessions/school/outing, Community Meeting) has concluded.
- The Afternoon Process meeting is attended by all members of the multidisciplinary team who are available at the time of the meeting.
- A minimum of two clinicians is required for the meeting to proceed. Clinicians who are unable to attend the Afternoon Process meeting are expected to provide a handover to one (or more) of the clinicians who will be in attendance at the Afternoon Process meeting. This handover must occur prior to the commencement of the meeting.
- Staff in attendance at the Afternoon Process meeting must provide an update of each client(s) presentation following their participation in the afternoon day program sessions.
- For each of their clients, Case Managers are responsible for documenting these discussions in the medical record (if the Case Manager is not available, this task is assigned to another clinician).
 - Individual therapy sessions, family therapy sessions, and Psychiatry reviews are documented separately by the clinician who facilitated the session. If deemed relevant by the facilitating clinician, each young person's presentation at these sessions may be discussed at the Afternoon Process meeting and documented in the medical record.
 - Community Meetings and Creative therapy sessions are not documented separately by the clinician who facilitated the session. Each young person's presentation at these sessions **must be** discussed at the afternoon process meeting and documented in the medical record by the young person's case manager.
- If the above process indicates that the young person's level of risk has increased, the young person's CRAMP must also be updated - See CAMHS [Risk Assessment and Management policy](#).

Mini team meetings

- A mini-team meeting can be called at any time by any member of the Touchstone MDT.
- The purpose of these meetings is to encourage Touchstone staff to collaborate and work as a team to assess, manage and treat the young people in their care.
- Although mini-team meetings are frequently called where there is concern about a young person's level of risk or engagement in the Touchstone program, they are not always risk-related and may be called in relation to any aspect of a young person's care. These meetings also help ensure that members of the Touchstone MDT are consistent in their approach to each young person's care.
- Mini team meetings must be documented in the outpatient notes section of the relevant young person's medical record, by one of the staff in attendance at the meeting.

Multidisciplinary Team (MDT) Clinical Review meetings

- MDT clinical review meetings are scheduled and documented as outlined in the [CAMHS MDT Clinical Reviews](#) policy.
- Additional ad hoc reviews or report writing reviews may also be scheduled as required. These must be documented in the outpatient notes section of the relevant young person's medical record, by one of the staff in attendance at the review.

Ad hoc meetings

- Staff may meet to discuss a client/s at any point throughout the day, particularly when there is a change in a young person's presentation or level of risk, or a crisis / incident.
- Any such discussions must be documented in the outpatient notes section of the young person's medical record, by one of the clinicians involved in the discussion. This should occur as soon as possible following the ad hoc discussion.

Re-entry meetings

- A re-entry meeting may be prompted by:
 - A breach of a non-negotiable behaviour (e.g. drug use in the program);
 - An act of violence (physical or verbal) towards self or others;
 - A crisis admission to hospital (note: a planned, collaboration admission would not prompt a re-entry meeting); or
 - A significant deviation from the agreed therapeutic contract.
- Following one of the above triggering events, a young person must not reintegrate back into the program until a re-entry meeting has occurred.

- The re-entry meeting should take place immediately after the triggering event, as soon as all required attendees are available to attend and engage in the meeting effectively. This includes the young person, parents/care givers, case manager; and co-case manager and/or senior clinician.
- A re-entry meeting is an active intervention that helps the young person makes sense of the triggering event and transition from impulse to self-regulation. The goals of the meeting are to:
 - Clarify events;
 - Identify and understand thoughts, feelings, needs and intentions;
 - Repair and restore relationships;
 - Teach new coping skills, problem solve issues and establish clear expectations moving forward;
 - Reintegrate the young person back into the Touchstone program; and
 - Review the young person's readiness to engage in the program in a meaningful way.
 - Reassess the level of risk and effectiveness of management strategies
- The Case Manager is responsible for:
 - Documenting the re-entry meeting discussions and outcomes in the medical record;
 - Completing an updated CRAMP;
 - Collaborating with the Touchstone leadership team to plan the re-entry meeting;
 - Liaising with Touchstone administrative staff to organise the meeting (include booking the appointment and room);
 - Informing the family of the next available date for the re-entry meeting (or delegating this task to another Touchstone staff member); and
 - Ensuring that all attendees have received (at minimum) verbal confirmation of the appointment time and place

Communication with parents and carers

- Pick up and drop off times are to be utilised as opportunities for staff to engage in two-way communication with parents/carers to ensure that issues, concerns and safety planning are effectively communicated.

Unaccompanied arrivals to and departures from Touchstone

- All young people attending the program must be dropped off and picked up by a parent/guardian or an adult aged 18 years or older as authorised by the parent/guardian.

- On rare occasions, due to unique extenuating circumstances, a young person may be authorised to arrive at or leave from Touchstone unaccompanied. This must be discussed and agreed by the HoS and HoD and brought to the attention/awareness of the Director of Clinical Services. Such exceptions must be approved in advance of unaccompanied arrivals/departures and will be based on the assessment of the capacity to travel independently. The evidence for this determination, including assessment of risk, must be documented in the medical file. A safety plan must also be developed with parents/caregivers and young person.
- Young people who leave the premises unauthorised and unaccompanied are to be managed in accordance with the CAHS Absconding/Missing Person's Policy. The young person's parent/guardian and consultant psychiatrist is to be notified. Staff are expected to not follow the young person beyond Touchstone's premises and to make all efforts to contact the young person and facilitate return to the premises or their home. The consultant psychiatrist in consultation with the parent/guardian and treatment team will review the most recent clinical assessment and level of risk to determine whether police involvement is required.
- Where a young person arrives at the premises unaccompanied without prior authorisation, staff will contact the parent/guardian to advise of the arrival, discuss the requirements for accompaniment and ensure the young person will be collected by an adult.
- Staff are to provide support to the parents/guardian of young people who are distressed, agitated or of concern at the time of leaving the premises. Staff must provide safety planning information and crisis contact details. Staff must not follow a parent who is following a young person in distress outside Touchstone premises.

Related CAHS internal policies, procedures and guidelines
CAHS Communicating for Safety Policy
CAMHS MDT Clinical Reviews
CAMHS Medical Records Management policy
CAMHS Risk Assessment and Management Policy
Crisis Incident Management in Touchstone Work Instruction

Useful resources (including related forms)
SSCD CAMHS Risk Assessment and Management Plan

This document can be made available in alternative formats on request.

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Standards Applicable:	NSQHS Standards:      Child Safe Principles: 1, 3, 6, 9, 10		
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